



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MIST RANDOM MOCK -2 DATED ON 11th MAY 2026

QUESTIONS

Q1. A 32-year-old man presents to the emergency department after a road traffic accident. He complains of difficulty in lifting his arm away from the body. On examination, he is unable to abduct the shoulder beyond 15°, although initiation of abduction is preserved. There is loss of sensation over the lateral aspect of the upper arm. Inspection reveals flattening of the shoulder contour with loss of deltoid prominence. X-ray shows a fracture at the surgical neck of the humerus.

Which of the following nerves is most likely injured in this patient?

- a) Radial nerve
- b) Musculocutaneous nerve
- c) Axillary nerve
- d) Suprascapular nerve

Q2. A 45-year-old carpenter presents with weakness of hand grip and difficulty in finger abduction and adduction after sustaining an injury near the medial epicondyle of the humerus. On examination, there is hyperextension at the metacarpophalangeal joints and flexion at the interphalangeal joints of the ring and little fingers. Froment's sign is positive.

Which of the following muscles is primarily responsible for this deformity?

- a) Lumbricals of index and middle finger
- b) Interossei muscles
- c) Thenar muscles
- d) Flexor digitorum superficialis

Q3. A 48-year-old woman presents with a painless lump in the upper outer quadrant of her right breast. Core biopsy confirms carcinoma. During surgical planning for modified radical mastectomy, the surgeon evaluates axillary lymph nodes in relation to the Pectoralis minor muscle to determine the stage and extent of spread.

Which group of axillary lymph nodes lies posterior to pectoralis minor?

- a) Level I nodes
- b) Level II nodes
- c) Level III nodes
- d) Apical nodes only

Q4. A 35-year-old woman presents with a malignant lesion located in the lateral quadrant of the breast. Sentinel lymph node biopsy is performed to assess regional spread. The involved lymph nodes are found to be situated along the lateral thoracic vessels. There is no evidence of distant metastasis.

Which group of axillary lymph nodes is most likely involved in this case?

- a) Anterior (pectoral) group
- b) Posterior (subscapular) group
- c) Lateral (humeral) group
- d) Central group

Q5. A 22-year-old athlete presents with recurrent shoulder dislocations following minor trauma. Imaging shows a shallow glenoid cavity with high mobility at the joint, allowing movements in multiple planes including rotation, abduction, and circumduction. The joint is classified as a synovial joint with maximum range of motion but poor stability.



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Which of the following best describes the type of joint involved?

- a) Hinge joint
- b) Pivot joint
- c) Saddle joint
- d) Ball and socket joint

Q6. A 14-year-old boy presents with pain and swelling around the knee after a sports injury. X-ray reveals involvement of the proximal tibial epiphysis, which contributes significantly to the longitudinal growth of the bone. The orthopedic surgeon explains that damage to this epiphysis may lead to growth disturbance.

Which type of epiphysis is most likely involved?

- a) Pressure epiphysis
- b) Traction epiphysis
- c) Atavistic epiphysis
- d) Aberrant epiphysis

Q7. A 35-year-old man presents after trauma to the infratemporal region with complaints of difficulty in chewing. On examination, there is weakness in elevation and retraction of the mandible. The muscle involved is identified as one of the muscles of mastication supplied by branches of the mandibular nerve. It is noted that this muscle has a dual embryological origin, contributing to its functional complexity.

Which of the following best describes this type of muscle?

- a) Composite muscle
- b) Hybrid muscle
- c) Pennate muscle
- d) Parallel muscle

Q8. A 55-year-old man presents with progressive dysphagia and hoarseness of voice. Imaging studies reveal a mediastinal mass compressing the esophagus and the left recurrent laryngeal nerve. The nerve is affected as it loops under the arch of the aorta before ascending in the tracheoesophageal groove. The lesion is located in the compartment that contains the arch of the aorta and its major branches.

Which part of the mediastinum is most likely involved?

- a) Superior mediastinum
- b) Anterior mediastinum
- c) Middle mediastinum
- d) Posterior mediastinum

Q9. A 40-year-old man presents with a stab injury to the lower thoracic region. He subsequently develops hoarseness, dysphagia, and impaired gastric motility. Imaging suggests injury to structures traversing the esophageal hiatus of the diaphragm at the level of T10. These structures are involved in autonomic innervation of the esophagus and stomach.

Which of the following structures passes through this opening?

- a) Inferior vena cava
- b) Thoracic duct
- c) Vagus nerves
- d) Azygos vein

Q10. A 60-year-old patient develops right shoulder pain following abdominal surgery complicated by a subphrenic abscess. On examination, there is irritation of the diaphragmatic pleura. The pain is localized to the shoulder region despite the pathology being intra-abdominal. This suggests referred pain due to shared neural pathways.



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Which of the following nerves is responsible for this referred pain?

- a) Intercostal nerves
- b) Vagus nerve
- c) Phrenic nerve
- d) Sympathetic trunk

Q11. A 52-year-old woman is brought for medico-legal autopsy with a history suggestive of neck compression. On external examination, there is bruising and contusion over the anterior aspect of the neck. Internal examination reveals fracture of the greater horn of the hyoid bone. There are no features suggestive of hanging such as an oblique ligature mark.

What is the most appropriate interpretation of this finding?

- a) It is commonly fractured in young individuals due to elasticity
- b) It indicates accidental fall injury to the neck
- c) It is suggestive of manual strangulation, especially in elderly
- d) It is pathognomonic of hanging

Q12. A 28-year-old male develops high-grade fever, severe headache, and swelling around the right eye following a facial infection in the “danger area” of the face. Examination reveals ophthalmoplegia, ptosis, and loss of corneal reflex. MRI confirms thrombosis of the Cavernous sinus.

Which of the following cranial nerves is most commonly affected first in this condition?

- a) Oculomotor nerve (CN III)
- b) Trochlear nerve (CN IV)
- c) Abducens nerve (CN VI)
- d) Ophthalmic division of trigeminal nerve (V1)

Q13. A 24-year-old woman presents with fever, headache, and periorbital swelling following a pustule over the upper lip. She is diagnosed with cavernous sinus thrombosis. It is suspected that the infection spread retrogradely from the “dangerous area of the face” through valveless venous channels.

Which of the following veins provides the most direct route for spread of infection to the cavernous sinus?

- a) External jugular vein
- b) Retromandibular vein
- c) Superior ophthalmic vein
- d) Internal jugular vein

Q14. A 62-year-old male presents with sudden onset of violent, involuntary, flinging movements of the right upper and lower limbs. There is no loss of consciousness. MRI reveals a small lacunar infarct in a deep gray matter structure of the left hemisphere supplied by perforating branches of the posterior cerebral artery.

Which of the following structures is most likely affected?

- a) Caudate nucleus
- b) Putamen
- c) Globus pallidus
- d) Subthalamic nucleus



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Q15. A 3-month-old infant presents with progressive increase in head size, bulging anterior fontanelle, and downward deviation of eyes ("sunset sign"). Imaging reveals dilation of lateral and third ventricles with a normal fourth ventricle.

Which of the following is the most likely site of obstruction?

- a) Foramen of Magendie
- b) Foramen of Luschka
- c) Cerebral aqueduct (Aqueduct of Sylvius)
- d) Interventricular foramen (Foramen of Monro)

Q16. A 32-year-old female presents with progressive muscle weakness that worsens toward the evening. She has difficulty in chewing and develops ptosis after prolonged reading. Repetitive nerve stimulation shows a decremental response. She is diagnosed with Myasthenia Gravis.

Which of the following is the primary physiological defect in this condition?

- a) Decreased release of acetylcholine from presynaptic terminal
- b) Blockade of postsynaptic acetylcholine receptors
- c) Destruction of muscle fibers
- d) Increased acetylcholinesterase activity

Q17. A 40-year-old male with a demyelinating disorder presents with slowed nerve conduction velocity and muscle weakness. Nerve conduction studies reveal impaired transmission along myelinated fibers.

Which of the following mechanisms is primarily affected in this patient?

- a) Continuous conduction along entire axon membrane
- b) Increased membrane capacitance
- c) Saltatory conduction between nodes of Ranvier
- d) Increased sodium leakage across axon membrane

Q18. A 50-year-old patient with chronic obstructive pulmonary disease develops chronic respiratory acidosis. Blood gas analysis shows elevated PaCO_2 with near-normal pH due to renal compensation.

Which of the following renal mechanisms is primarily responsible for this compensation?

- a) Decreased bicarbonate reabsorption
- b) Increased excretion of bicarbonate
- c) Increased hydrogen ion secretion and bicarbonate generation
- d) Decreased ammonium production

Q19. A 60-year-old chronic smoker presents with progressive dyspnea. Arterial blood gas shows hypoxemia with normal PaCO_2 . Imaging suggests areas of lung receiving adequate perfusion but poor ventilation. The physiologist explains that this leads to a low V/Q ratio in affected regions.

Which of the following best describes the physiological consequence of a low V/Q ratio?

- a) Increased alveolar PO_2 and decreased PCO_2
- b) Decreased alveolar PO_2 and increased PCO_2
- c) No change in gas exchange
- d) Increased oxygen diffusion capacity



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Q20. A 28-year-old man with high fever and sepsis presents with tachypnea and hypoxia. Laboratory findings reveal metabolic acidosis and increased 2,3-BPG levels. How will these changes affect the oxygen–hemoglobin dissociation curve?

- a) Shift to left with increased affinity for oxygen
- b) Shift to right with decreased affinity for oxygen
- c) No change in oxygen affinity
- d) Decreased oxygen delivery to tissues

Q21. A preterm neonate develops respiratory distress shortly after birth. Chest X-ray shows ground-glass appearance. The condition is attributed to deficiency of pulmonary surfactant.

What is the primary physiological role of surfactant in this scenario?

- a) Increase alveolar surface tension
- b) Decrease lung compliance
- c) Reduce alveolar surface tension and prevent collapse
- d) Increase airway resistance

Q22. A 30-year-old marathon runner collapses after excessive water intake during a race. He presents with confusion and seizures. Laboratory findings reveal low serum sodium and decreased plasma osmolality.

Which of the following changes is most likely occurring in his body fluid compartments?

- a) Decreased intracellular fluid (ICF) volume
- b) Increased extracellular fluid (ECF) osmolality
- c) Water movement from ECF to ICF causing cellular swelling
- d) No change in cell volume

Q23. A 5-year-old child presents with glucose-galactose malabsorption leading to severe diarrhea. The defect lies in a transporter responsible for glucose absorption in the intestine.

Which of the following mechanisms best describes this transport process?

- a) Simple diffusion
- b) Facilitated diffusion
- c) Primary active transport
- d) Secondary active transport (Na^+ -glucose cotransport)

Q24. A 40-year-old patient develops muscle weakness due to hyperkalemia. Laboratory tests show elevated extracellular potassium levels.

How will this condition affect the resting membrane potential (RMP) of skeletal muscle cells?

- a) Hyperpolarization of membrane
- b) Depolarization of membrane
- c) No change in membrane potential
- d) Increased action potential threshold

Q25. A 45-year-old male presents with recurrent epigastric pain that worsens at night and is relieved by food. Endoscopy suggests a duodenal ulcer. He is started on a proton pump inhibitor that irreversibly inhibits the final step of acid secretion in parietal cells.

Which of the following is the direct target of this drug?

- a) Histamine H_2 receptor



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- b) Gastrin receptor
- c) H^+/K^+ ATPase pump
- d) Carbonic anhydrase enzyme

Q26. A 35-year-old patient consumes a fatty meal. Soon after, pancreatic enzyme secretion increases significantly. This response is mediated by a hormone released from the duodenal mucosa.

Which of the following hormones is primarily responsible?

- a) Gastrin
- b) Secretin
- c) Cholecystokinin (CCK)
- d) Somatostatin

Q27. A 28-year-old woman presents with complaints of fatigue, pallor, and decreased exercise tolerance. Laboratory investigations reveal microcytic hypochromic anemia consistent with iron deficiency. She is advised oral iron supplementation along with dietary modifications. The physician explains the site of maximal iron absorption in the gastrointestinal tract to optimize therapy.

In which part of the gastrointestinal tract is iron primarily absorbed?

- a) Stomach
- b) Duodenum
- c) Jejunum
- d) Ileum

Q28. A 55-year-old man with chronic anemia presents with exertional dyspnea and fatigue. On evaluation, his cardiac output is found to be elevated as a compensatory mechanism for reduced oxygen-carrying capacity. The increase in stroke volume is attributed to enhanced ventricular filling, leading to stronger myocardial contraction. The physician explains that this mechanism depends on the intrinsic property of cardiac muscle fibers.

Which physiological principle best explains this phenomenon?

- a) Bowditch effect
- b) Frank-Starling law of the heart
- c) Bainbridge reflex
- d) Baroreceptor reflex

Q29. A 60-year-old patient undergoes cardiac catheterization for evaluation of cardiac function. Atrial pressure tracings are recorded during the cardiac cycle. The tracing shows a prominent “v wave” occurring during ventricular systole. The physician explains that this wave reflects changes in atrial pressure while the atrioventricular (AV) valves remain closed.

Which of the following events corresponds to the “v wave”?

- a) Atrial contraction
- b) Closure of AV valves
- c) Passive filling of atrium against closed AV valve
- d) Opening of semilunar valves

Q30. A 50-year-old man with long-standing hypertension is evaluated for increased systemic vascular resistance. The physician explains that vascular tone plays a crucial role in regulating blood flow. Even minor



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changes in vessel diameter can lead to significant alterations in resistance. This relationship is governed by Poiseuille's law.

Which of the following changes will have the greatest impact on vascular resistance?

- a) Doubling the vessel length
- b) Doubling blood viscosity
- c) Halving the vessel radius
- d) Doubling the pressure gradient

Q31. A 6-year-old child presents with recurrent episodes of hypoglycemia, muscle weakness, and lethargy, especially during fasting. Laboratory findings show low ketone bodies despite hypoglycemia. A defect in transport of long-chain fatty acids into mitochondria is suspected.

Which of the following is most likely deficient in this patient?

- a) Acetyl-CoA carboxylase
- b) Carnitine
- c) Hormone-sensitive lipase
- d) Fatty acid synthase

Q32. A 40-year-old obese male presents with hypertriglyceridemia. Laboratory evaluation shows elevated chylomicrons and VLDL. The defect is attributed to impaired activation of lipoprotein lipase (LPL).

Which apolipoprotein is required for activation of LPL?

- a) Apo A-I
- b) Apo B-100
- c) Apo C-II
- d) Apo E

Q33. A 55-year-old man with known coronary artery disease is initiated on statin therapy to reduce cardiovascular risk. The drug acts by inhibiting the rate-limiting step in hepatic cholesterol synthesis. This leads to decreased intracellular cholesterol levels and upregulation of LDL receptors, thereby lowering plasma LDL cholesterol.

Which of the following enzymes is inhibited by this drug?

- a) HMG-CoA synthase
- b) HMG-CoA reductase
- c) Acetyl-CoA carboxylase
- d) Cholesterol esterase

Q34. A 3-day-old newborn is brought with bleeding from the umbilical stump and episodes of gastrointestinal bleeding. The baby was born at home, and no antenatal or postnatal vitamin supplementation was given. The infant is exclusively breastfed. Laboratory investigations reveal a prolonged prothrombin time (PT), indicating a defect in coagulation factor synthesis.

Deficiency of which of the following vitamins is most likely responsible for this condition?

- a) Vitamin A
- b) Vitamin D
- c) Vitamin K
- d) Vitamin E



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Q35. A 45-year-old chronic alcoholic presents with confusion, gait instability, and abnormal eye movements. Clinical examination reveals ataxia and ophthalmoplegia, and a diagnosis of Wernicke encephalopathy is made. The condition is attributed to deficiency of a vitamin essential for carbohydrate metabolism. This vitamin acts as a cofactor for key enzymes involved in oxidative decarboxylation reactions.

Which of the following enzymes requires this vitamin as a cofactor?

- a) Pyruvate dehydrogenase
- b) Lactate dehydrogenase
- c) Glucose-6-phosphate dehydrogenase
- d) Hexokinase

Q36. A 60-year-old strict vegetarian presents with fatigue, glossitis, and progressive numbness in the extremities. Laboratory evaluation reveals macrocytic anemia with hypersegmented neutrophils. Neurological examination shows impaired vibration and position sense. The findings suggest a deficiency affecting both hematological and neurological systems.

Which of the following best explains the neurological symptoms in this patient?

- a) Impaired purine synthesis
- b) Defective methylmalonyl-CoA metabolism
- c) Reduced iron absorption
- d) Increased homocysteine clearance

Q37. A middle-aged man with a history of chronic alcohol consumption presents with fatigue, malaise, and vague abdominal discomfort. Laboratory investigations reveal a significant elevation in serum gamma-glutamyl transferase (GGT) levels. Other liver function parameters may also be mildly deranged. The physician explains the diagnostic significance of this enzyme in relation to alcohol use and liver pathology.

What is the clinical significance of elevated GGT in this patient?

- a) Marker of myocardial injury
- b) Marker of hepatobiliary disease and alcohol abuse
- c) Marker of skeletal muscle injury]
- d) Marker of pancreatic injury

Q38. A young child is brought with developmental delay and recurrent episodes of self-mutilation, including biting of lips and fingers. Caregivers also report orange-colored crystals in the diaper. Laboratory investigations reveal markedly elevated serum uric acid levels. The clinical picture suggests a disorder of purine metabolism affecting salvage pathways.

Which of the following enzyme deficiencies is responsible for this condition?

- a) Xanthine oxidase
- b) Adenosine deaminase
- c) HGPRT
- d) PRPP synthetase

Q39. A factory worker presents with sudden onset headache, dizziness, and shortness of breath after accidental inhalation of cyanide gas. Blood oxygen levels are normal but tissues show signs of severe hypoxia. Cyanide inhibits the final step of the electron transport chain, preventing ATP synthesis despite adequate oxygen availability.



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Which component of the ETC is inhibited?

- a) NADH dehydrogenase
- b) Succinate dehydrogenase
- c) Cytochrome oxidase
- d) ATP synthase

Q40. A 58-year-old man presents to the emergency department with severe chest pain radiating to the left arm. ECG shows ST-segment elevation. Laboratory investigations reveal elevated cardiac enzyme levels. The physician orders a biomarker that is highly specific for myocardial injury and remains elevated for several days after infarction.

Which enzyme/protein is most specific for diagnosing myocardial infarction?

- a) Lactate dehydrogenase
- b) Creatine kinase-MB
- c) Troponin I
- d) Aspartate aminotransferase

Q41. A 22-year-old marathon runner collapses during a race after prolonged fasting. He complains of muscle cramps and weakness. Laboratory tests show low blood glucose and elevated lactate levels after exertion. A muscle biopsy reveals accumulation of glycogen with deficiency of an enzyme responsible for glycogen breakdown in skeletal muscle. The patient reports exercise intolerance since childhood and occasionally passes dark colored urine after strenuous activity.

Which enzyme deficiency is most likely responsible?

- a) Glucose-6-phosphatase
- b) Muscle glycogen phosphorylase
- c) Debranching enzyme
- d) Branching enzyme

Q42. A child presents with recurrent infections and features suggestive of severe immunodeficiency. Laboratory evaluation reveals defective DNA synthesis due to impaired conversion of ribonucleotides into deoxyribonucleotides. This defect affects rapidly dividing cells, particularly lymphocytes, leading to compromised immune function.

Which of the following enzymes catalyzes this crucial step in nucleotide metabolism?

- a) Thymidylate synthase
- b) Ribonucleotide reductase
- c) Adenosine deaminase
- d) Xanthine oxidase

Q43. A 45-year-old man presents with a history of progressively enlarging nodular swellings over his Achilles tendon. He has a strong family history of premature coronary artery disease. On examination, tendon xanthomas are noted. Laboratory investigations reveal markedly elevated LDL cholesterol levels with normal triglycerides.

Which of the following genetic defects is most likely responsible for this condition?

- a) Lipoprotein lipase deficiency
- b) LDL receptor mutation
- c) Apo C-II deficiency



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d) LCAT deficiency

Q44. A patient presents with recurrent episodes of flushing, diarrhea, and wheezing. Further evaluation reveals a carcinoid tumor secreting excessive amounts of serotonin. Laboratory tests show elevated levels of serotonin metabolites in urine. The physician explains the biochemical pathway involved in the synthesis of this amine.

Serotonin is synthesized from which of the following amino acids?

- a) Tyrosine
- b) Tryptophan
- c) Phenylalanine
- d) Histidine

Q45. A healthy individual consumes a carbohydrate-rich meal. A few hours later, he is in the absorptive (fed) state, during which nutrients from the gastrointestinal tract are actively being utilized and stored. Hormonal changes favor anabolic processes, and various metabolic parameters shift accordingly.

Which of the following is elevated in plasma during the absorptive state compared to the postabsorptive state?

- a) Glucagon
- b) Acetoacetate
- c) Chylomicrons
- d) Free fatty acids

Q46. A 38-year-old woman presents with intermittent pelvic pain and is found to have a 3-cm ovarian mass on imaging. Surgical excision is performed, and histopathological examination reveals a papillary tumor with multiple small, round, laminated calcified structures. These calcifications are characteristic and help in narrowing the diagnosis.

Which of the following statements regarding these calcifications is NOT true?

- a) Can be stained with Von Kossa stain
- b) Result of mutation in macrophages
- c) Serum calcium is normal
- d) They represent dystrophic calcification

Q47. A 3-year-old boy presents with recurrent bacterial and fungal infections involving the skin and respiratory tract. On examination, he has oculocutaneous albinism. Peripheral blood smear shows giant granules within neutrophils, lymphocytes, and monocytes. Laboratory studies reveal decreased neutrophil count and impaired bactericidal function due to defective fusion of phagosomes with lysosomes.

What is the most likely diagnosis?

- a) Ataxia-telangiectasia
- b) Chédiak-Higashi syndrome
- c) Chronic granulomatous disease
- d) Ehlers-Danlos syndrome

Q48. A 27-year-old woman presents with progressive visual disturbances. On examination, she is unusually tall and thin with long, slender fingers (arachnodactyly). Ophthalmic evaluation reveals displacement of the lens into the anterior chamber. Laboratory tests show normal methionine and cystathionine levels. The clinical picture suggests a genetic disorder affecting connective tissue.

Which chromosome is most commonly involved in this condition?



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- a) 21
- b) 13

- c) 15
- d) 18

Q49. A routine chest X-ray in an asymptomatic adult male working in sandblasting shows fine nodular opacities predominantly in the zones of both lungs. There is characteristic “eggshell” calcification of the hilar lymph nodes. Lung biopsy demonstrates birefringent particles within macrophages. The patient has normal serum calcium and total protein levels, with no history of smoking or drug use.

Which of the following statements regarding this condition is NOT true?

- a) The patient is suffering from silicosis
- b) It commonly involves the upper lobes
- c) There would be high risk of tuberculosis
- d) It is the most common pneumoconiotic disease

Q50. A male neonate dies within 24 hours of birth. Autopsy reveals multiple congenital anomalies, including polydactyly, cleft lip and palate, and a single central eye (cyclopia). Brain examination shows holoprosencephaly with fused frontal lobes and a single ventricular cavity. These findings suggest a severe chromosomal abnormality with midline defects.

Which of the following chromosomal abnormalities is most consistent with this presentation?

- a) Deletion 21
- b) Deletion 22
- c) Trisomy 13
- d) Trisomy 18

Q51. A patient with advanced malignancy presents with progressive weight loss, muscle wasting, and generalized weakness despite adequate nutritional intake. Similar features are observed in patients with chronic infections such as tuberculosis and AIDS. This condition is termed cachexia and is mediated by inflammatory cytokines affecting metabolism and appetite.

Which of the following factors is most likely responsible for this syndrome?

- a) Interleukin-2
- b) Interleukin-4
- c) Tumor necrosis factor
- d) Interferon

Q52. A patient presents with fever and right-sided cervical lymphadenopathy. Biopsy of the lymph node reveals a background of normal lympho-mononuclear cells with scattered large atypical cells. Immunohistochemistry shows these cells to be CD20 positive and negative for CD15, CD30, and EBV latent membrane protein.

Which of the following is the most likely diagnosis?

- a) Lymphocyte-rich Hodgkin lymphoma
- b) Diffuse large B-cell lymphoma
- c) Nodular lymphocyte predominant Hodgkin lymphoma
- d) Small cell lymphoma

Q53. A 20-year-old boy presents with complaints of gum bleeding, easy bruising, and fever for the past one month. On examination, he is pale and has multiple petechial rashes over the body. Laboratory investigations



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reveal hemoglobin of 3 g/dL, total leukocyte count of 15,000/cmm, and platelet count of 15,000/cmm. Bone marrow examination shows a hypocellular marrow without the presence of immature (blast) cells.

What is the most probable diagnosis?

- a) Acute lymphoblastic leukemia (ALL)
- b) Aplastic anemia
- c) Paroxysmal nocturnal hemoglobinuria
- d) None of the above

Q54. A patient undergoing evaluation for a hepatic mass is referred for nuclear imaging studies. The clinician explains that certain liver lesions can be identified with high accuracy based on their uptake of radiotracers due to the presence of functioning Kupffer cells. The imaging findings help differentiate benign from malignant lesions.

Which of the following hepatic lesions can be diagnosed with high accuracy using nuclear imaging?

- a) Hepatocellular carcinoma
- b) Hepatic adenoma
- c) Focal nodular hyperplasia
- d) Cholangiocarcinoma

Q55. A patient diagnosed with hepatitis B infection undergoes serological testing. His laboratory report shows the presence of HBeAg in the serum. The clinician explains that this marker reflects viral replication and infectivity.

What does the presence of HBeAg in serum indicate?

- a) Acute non-infectious stage
- b) Acute, infectious state
- c) Chronic, non-infectious stage
- d) Chronic, infectious stage

Q56. A patient with a history of vesicoureteral reflux is diagnosed with reflux nephropathy on imaging. Over time, he develops significant proteinuria in the nephritic range. There is no history of diabetes or systemic autoimmune disease. Renal biopsy is planned to evaluate the underlying glomerular pathology.

Which of the following histological findings is most likely in this patient?

- a) Nodular glomerulosclerosis
- b) Focal segmental glomerulosclerosis
- c) Membranous glomerulopathy
- d) Proliferative glomerulonephritis with crescents

Q57. A patient presents with cough, hemoptysis, and imaging reveals multiple cavitary lesions in the lungs. He also has hematuria with red cell casts on urine examination, indicating glomerular involvement. There is no history of asthma or allergic disease. The clinical picture suggests a systemic vasculitic disorder involving both respiratory tract and kidneys.

What is the most likely diagnosis?

- a) Anti-GBM disease
- b) Churg-Strauss allergic granulomatosis
- c) Systemic lupus erythematosus
- d) Wegener's granulomatosis



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Q58. A chronic smoker presents with progressive dyspnea and reduced exercise tolerance. Imaging shows hyperinflated lungs with areas of airspace enlargement predominantly affecting the upper lobes. The pathology involves selective destruction of a specific part of the acinus.

Which of the following is the correct association regarding the pathogenesis of smoking-induced emphysema?

- a) Destruction of distal acinus = centrilobular emphysema
- b) Destruction of distal acinus = paraseptal emphysema
- c) Destruction of entire acinus = panlobular emphysema
- d) Destruction of proximal acinus = centrilobular emphysema

Q59. A tall adolescent male presents with gynecomastia. On examination, he has underdeveloped secondary sexual characteristics, including absence of deep voice, sparse facial hair, and lack of typical male-pattern pubic hair distribution. Cognitive function is within normal limits. Genetic evaluation reveals the presence of a Barr body.

What is the most likely diagnosis in this patient?

- a) Turner syndrome
- b) Klinefelter syndrome
- c) Swyer syndrome
- d) Down syndrome

Q60. A 59-year-old male presents with a 3.5-cm mass in the right upper lobe of the lung. Biopsy confirms moderately differentiated squamous cell carcinoma. Further evaluation shows no evidence of bone metastasis. However, laboratory findings reveal elevated serum calcium levels (11.5 mg/dL). The clinician suspects a paraneoplastic syndrome as the cause of hypercalcemia.

Which of the following substances is most likely responsible for this finding?

- a) Parathyroid hormone (PTH)
- b) Parathyroid hormone-related peptide (PTHrP)
- c) Calcitonin
- d) Calcitonin-related peptide

Q61. A woman with estrogen receptor-positive breast carcinoma is started on tamoxifen therapy. During follow-up, the clinician counsels her regarding potential adverse effects of the drug, which are related to its selective estrogen receptor modulator (SERM) activity. Some tissues experience anti-estrogenic effects, while others have partial agonist effects.

Which of the following is NOT a known side effect of tamoxifen therapy?

- a) Thromboembolic events
- b) Endometrial carcinoma
- c) Cataract
- d) Cancer in opposite breast

Q62. A known diabetic patient with poor glycemic control on metformin and glimepiride is started on an additional oral hypoglycemic agent. Within one month, his blood glucose levels improve significantly. However, he presents with recurrent episodes of urinary tract infections (UTIs). There is no prior history of recurrent infections. Laboratory findings are otherwise unremarkable.

Which of the following drugs is most likely responsible for this presentation?



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- a) Pioglitazone
- b) Acarbose
- c) Canagliflozin
- d) Liraglutide

Q63. A 30-year-old pregnant woman with a history of rheumatoid arthritis has been on long-term NSAID therapy. She now presents with burning epigastric pain that worsens after meals. Clinical evaluation suggests NSAID-induced gastritis or peptic ulcer disease. The physician plans to initiate anti-ulcer therapy but must consider drug safety in pregnancy.

Which of the following medications is contraindicated in this patient?

- a) Famotidine
- b) Omeprazole
- c) Misoprostol
- d) Ranitidine

Q64. A patient diagnosed with meningioma presents with complaints of progressive hearing difficulty and imbalance. MRI reveals significant edema in the temporal lobe region causing mass effect. The patient has features of raised intracranial pressure, and immediate symptomatic management is planned to reduce cerebral edema.

Which of the following drugs is most appropriate for initial symptomatic relief in this patient?

- a) Acetazolamide
- b) Furosemide
- c) Mannitol
- d) Amiloride

Q65. A patient presents with acute onset diarrhea suspected to be infective in origin. The physician considers symptomatic management along with supportive care. While selecting medications, the role and safety of antimotility drugs are evaluated carefully, especially in infectious conditions.

Which of the following statements regarding treatment of diarrhea is NOT correct?

- a) Anti-motility drugs are the drug of choice for infective diarrhea
- b) Loperamide and diphenoxylate are opioids used as anti-motility drugs
- c) Diphenoxylate overdose can cause respiratory depression
- d) Opioids decrease peristalsis and can be used in diarrhea

Q66. A 30-year-old woman undergoing chemotherapy for ovarian cancer presents with progressive hearing loss, particularly affecting high-frequency sounds. She also develops worsening renal function during treatment. There is no prior history of ear disease or kidney disorder. The treating oncologist suspects drug-induced toxicity.

Which of the following drugs is most likely responsible for this presentation?

- a) Bleomycin
- b) Cisplatin
- c) Doxorubicin
- d) 5-fluorouracil



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Q67. A patient with bronchial asthma is prescribed an inhaled bronchodilator for both rapid relief of acute symptoms and long-term maintenance therapy. The physician prefers a drug with a rapid onset of action as well as a long duration, allowing it to be used for both relief and prophylaxis when given twice daily.

Which of the following β_2 -agonists best fits this profile?

- a) Terbutaline
- b) Bambuterol
- c) Salmeterol
- d) Formoterol

Q68. A patient is brought to the emergency department with complaints of high fever, palpitations, and altered behavior. On examination, he has tachycardia, hyperthermia, dry skin, dilated bronchi, and decreased bowel movements leading to constipation. Pupils are dilated and mucous membranes appear dry. The clinical picture suggests a toxidrome affecting parasympathetic pathways.

Which of the following is the most likely cause of this presentation?

- a) Atropine overdose
- b) Organophosphorus compound poisoning
- c) Mushroom poisoning
- d) Paracetamol overdose

Q69. A patient in the ICU is started on a sedative/antipsychotic medication for agitation and delirium. Within a few days, he develops high-grade fever, severe muscle rigidity, altered mental status, and autonomic instability. Laboratory investigations reveal elevated creatine phosphokinase (CPK) levels. The clinical picture suggests a life-threatening drug reaction.

Which of the following drugs is most likely responsible for this condition?

- a) Midazolam
- b) Lorazepam
- c) Diazepam
- d) Haloperidol

Q70. A 30-year-old man presents with repetitive, involuntary movements characterized by excessive blinking and intermittent grunting sounds. These symptoms have been present for several months and are increasing in frequency. He reports inability to suppress these movements, which interfere with his daily functioning. There is no history of seizures or mood disorder. The clinical picture is suggestive of a tic disorder.

Which of the following medications is most appropriate for treatment?

- a) Carbamazepine
- b) Imipramine
- c) Risperidone
- d) Methylphenidate

Q71. A diabetic patient presents with cellulitis involving the lower limb. Culture and sensitivity testing reveals *Staphylococcus aureus* resistant to methicillin (MRSA). The clinician plans to initiate appropriate antibiotic therapy targeting resistant gram-positive organisms.

Which of the following antibiotics is NOT appropriate for treating this infection?

- a) Vancomycin
- b) Imipenem



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- c) Teicoplanin
- d) Linezolid

Q72. A 32-year-old woman presents to the gynecology outpatient department with complaints of progressively increasing menstrual bleeding over the past few months. She reports using 4–5 sanitary pads per day, with bleeding lasting more than 7 days. There is no history of pregnancy or bleeding disorders. Clinical evaluation suggests menorrhagia, and medical management is planned.

Which of the following is the most appropriate drug for this patient?

- a) Mefenamic acid
- b) Tranexamic acid
- c) Vitamin K
- d) Acetylsalicylic acid

Q73. A 50-year-old man presents with sudden onset of bizarre behavior, confusion, and altered mental status. There is no history of trauma or substance abuse. CSF analysis shows lymphocytic pleocytosis. MRI brain reveals abnormalities predominantly involving the temporal lobe. The clinician suspects a viral etiology affecting the limbic system.

Which of the following is the most likely causative organism?

- a) Herpes simplex virus type 1
- b) Herpes simplex virus type 2
- c) Cocksackievirus B
- d) Cryptococcus neoformans

Q74. A patient presents with acute infection, and rapid detection of microbial antigen is required for early diagnosis and management. The laboratory uses a test in which inert particles are coated with antigen or antibody, leading to visible agglutination within minutes. The clinician prefers a method that provides results in less than 5 minutes.

Which of the following tests is most appropriate for this purpose?

- a) Coagglutination (COA)
- b) Counterimmunoelectrophoresis (CIE)
- c) Enzyme-linked immunosorbent assay (ELISA)
- d) Latex agglutination

Q75. A patient from Assam presents with intermittent fever and chills for the past two weeks. On examination, he appears pale. A rapid antigen test for malaria is performed and is found to be positive for HRP-2 antigen. The clinician considers the species of Plasmodium responsible for this infection.

Which of the following species is most likely causing the disease?

- a) Falciparum
- b) Malariae
- c) Ovale
- d) Vivax

Q76. An HIV-positive patient presents with chronic diarrhea persisting for the past 6 months. He has significant weight loss and dehydration. Stool examination reveals cyst-like structures that are positive on



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modified Kinyoun acid-fast staining. The clinician suspects an opportunistic protozoal infection commonly seen in immunocompromised individuals.

What is the most likely diagnosis?

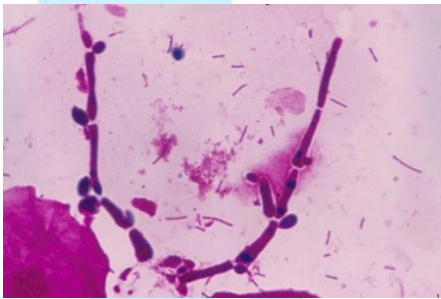
- a) Balantidium coli
- b) Clostridium difficile
- c) Escherichia coli
- d) Cryptosporidium

Q77. A patient presents to the emergency department with symptoms of pneumonia, including fever, dyspnea, and cough. Imaging suggests diffuse interstitial involvement. Laboratory evaluation includes special staining of respiratory samples, which reveals black-colored cysts on Gomori methenamine silver (GMS) stain. The clinician suspects an opportunistic pathogen.

What is the most likely causative organism?

- a) Cryptococcus
- b) Pneumocystis
- c) Coccidioides
- d) Paracoccidioides

Q78. An AIDS patient presents with oral thrush and dysphagia. Gram stain of oropharyngeal scrapings reveals budding yeast cells along with pseudohyphae. The organism is suspected to be a common opportunistic pathogen in immunocompromised individuals. Under which morphological category is this organism classified?



- a) Yeast
- b) Yeast-like fungus
- c) Dimorphic fungus
- d) Mold

Q79. A pregnant woman presents with cervical lymphadenopathy and is diagnosed with a parasitic infection. She is prescribed spiramycin but is non-compliant with treatment. The newborn is later found to have intracerebral calcifications on imaging. There may also be associated neurological deficits.

Which of the following is the most likely diagnosis?

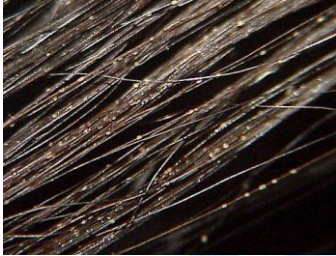
- a) Toxoplasma
- b) Cytomegalovirus
- c) Cryptococcus
- d) Rubella



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Q80. A 25-year-old woman presents with multiple white nodular lesions over her scalp hair noticed over the past 2 weeks. On examination, the nodules are soft and completely encircle the hair shaft. Microscopy confirms fungal elements, and culture grows yellowish to cream-colored fluffy colonies. Which of the following organisms is the most likely cause of this condition?



- a) *Piedraia hortae*
- b) *Pityrosporum orbiculare*
- c) *Hortaea werneckii*
- d) *Trichosporon beigeli*

Q81. A 2-year-old child attending daycare presents with acute onset diarrhea, vomiting, and features of gastroenteritis. Several other children in the daycare have similar symptoms. Laboratory evaluation suggests a viral etiology, and adenovirus infection is suspected.

Which of the following adenoviral serotypes is most commonly associated with this presentation?

- a) Type 4
- b) Type 7
- c) Type 19
- d) Type 41

Q82. A child presents with fever, respiratory distress, and a thick white patch over the tonsils. On examination, a pseudomembrane is noted, raising suspicion of diphtheria. A throat swab is collected for microbiological confirmation. The laboratory plans to culture the organism on a suitable medium that enhances its growth and characteristic morphology.

Which of the following culture media is most appropriate for this purpose?

- a) Lowenstein-Jensen (LJ) medium
- b) Cary-Blair medium
- c) Loeffler serum slope
- d) Robertson's cooked meat medium

Q83. A 24-year-old woman presents with sudden onset oligomenorrhea, excessive facial hair growth, and progressive development of masculine body features. She also reports voice deepening and increased muscle mass over a short duration. There is no history of childbirth or postpartum complications. The rapid progression of symptoms suggests a hormonally active pathology.

What is the most likely diagnosis?

- a) Polycystic ovarian disease
- b) Sheehan syndrome
- c) Stein-Leventhal syndrome
- d) Androblastoma



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Q84. A patient presents with acute onset fever, productive cough with rusty brown sputum, and pleuritic chest pain. On examination, signs of lung consolidation are noted. Laboratory testing shows a positive Quellung reaction, confirming the presence of a capsulated organism. The pathogen is cultured on appropriate media for identification.

What is the most characteristic appearance of this organism on culture?

- a) Golden yellow colonies
- b) Carrom-coin colonies
- c) Oil-paint colonies
- d) Black color colonies

Q85. A body is recovered from a water body and brought for postmortem examination. The forensic expert looks for signs that can reliably indicate drowning as the cause of death. On examination, certain external findings are noted, but only one of them is considered a sure (pathognomonic) sign of drowning.

Which of the following is a sure external sign of drowning?

- a) Goose skin
- b) Washerwoman's hands
- c) Peeling of skin
- d) Fine froth at mouth and nostrils

Q86. A 35-year-old male is brought for medicolegal autopsy after being found dead in his room with a firearm nearby. On examination, there is a single entry wound over the temple. The wound margins are irregular, and there is blackening and burning of surrounding skin. Notably, a distinct circular patterned mark. On further dissection, there is evidence of soot deposition in deeper tissues. The forensic expert concludes that the shot was fired with the muzzle in direct contact with the skin.

What is the most important sign for identifying a contact firearm entry wound?

- a) Loss of substance
- b) Presence of two wounds
- c) Muzzle imprint around the wound margin
- d) Abraded inlet

Q87. A 19-year-old male presents with bleeding gums, easy bruising, and poor wound healing. He also complains of joint pain and fatigue. On examination, there are perifollicular hemorrhages and corkscrew hairs. Laboratory findings suggest defective collagen synthesis. The treating physician suspects a nutritional deficiency affecting post-translational modification of collagen. During a teaching discussion, the professor asks about the structural and biochemical properties of collagen, including its molecular organization and synthesis.

Which of the following statements regarding collagen is incorrect?

- a) Triple helical structure is a right-handed superhelix
- b) Glycine residue at every 3rd position of α chains
- c) Vitamin C is necessary during post-translational modification
- d) Tropocollagen molecules are supramolecular assemblies that finally attain a left-handed super super coil

Q88. A 26-year-old married woman presents to the gynecology OPD at 9 weeks of gestation requesting termination of pregnancy due to personal reasons. She is mentally sound and understands the procedure and its



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implications. Her husband is currently abroad and cannot be contacted. The attending doctor explains the procedure, risks, and alternatives. The patient gives written informed consent for termination. However, the nursing staff raises a concern about whether the husband's consent is also required before proceeding under the Medical Termination of Pregnancy Act.

Whose consent is legally required in this situation?

- a) Husband of the lady
- b) Both husband and wife
- c) Concerned lady
- d) Guardian

Q89. A 32-year-old male factory worker is brought to the emergency department after an accidental explosion in a fireworks manufacturing unit. He has superficial burns over his hands and face, along with black soot deposition on his clothes. On investigation, the forensic team recovers a partially burnt powder mixture from the site. Laboratory analysis suggests the presence of potassium nitrate, charcoal, and sulfur in specific proportions typical of traditional explosive material. The forensic expert is asked to identify the composition of the substance, commonly referred to as black powder or gunpowder, used in low-order explosives.

What is the correct composition of black powder (gunpowder)?

- a) K-nitrate 5%, charcoal 5%, sulphur 10%
- b) K-nitrate 7%, charcoal 1%, sulphur 1%
- c) K-nitrate 15%, charcoal 15%, sulphur 15%
- d) K-nitrate 75%, charcoal 15%, sulphur 10%

Q90. A 40-year-old male is brought for medicolegal autopsy after a firearm injury during a street altercation. On examination, the entry wound over the chest is oval and irregular, resembling a "keyhole" appearance with an abraded margin on one side. The forensic expert notes that such wounds occur when the bullet does not strike the body in its usual nose-first orientation. Further analysis suggests that the projectile had an unstable flight and struck the body sideways rather than in a straight trajectory.

Which type of bullet behavior is most likely responsible for this type of entry wound?

- a) Ricochet bullet
- b) Yawing bullet
- c) Tumbling bullet
- d) Tandem bullet

Q91. A 28-year-old male is brought to the emergency department after sustaining a firearm injury during a violent encounter. On examination, there is a large, irregular entry wound over the abdomen with extensive surrounding tissue destruction. Surgical exploration reveals massive cavitation and fragmentation of tissues along the wound track. Radiological imaging shows multiple small metallic fragments dispersed within the soft tissues rather than a single intact projectile. The forensic expert suspects the use of a special type of bullet designed to expand on impact and cause maximum damage.



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Identify the type of bullet most likely responsible for these findings.

- a) Glancing bullet
- b) Dum Dum bullet
- c) Tracer bullet
- d) Tandem bullet

Q92. A 7-year-old child is brought to the emergency department after being run over by an auto-rickshaw while playing on the roadside. On examination, there is a reddish-blue injury over the lateral aspect of the thigh. The lesion shows a clear pattern resembling the tread of a tyre, with alternating linear and grooved impressions corresponding to the vehicle's tyre design. There is no break in the skin, but the patterned discoloration is sharply demarcated. The forensic expert is asked to classify the nature of this injury based on its appearance and mechanism.

What is the most appropriate classification of this injury?

- a) Contact bruise
- b) Patterned bruise
- c) Imprint abrasion
- d) Ectopic bruise

Q93. A 30-year-old male is found dead in a semi-sitting position with a ligature tied to a ceiling hook. His feet are touching the ground, and knees are slightly flexed. On examination, a well-defined ligature mark is seen high up on the neck, with features suggestive of antemortem hanging. Police report indicates partial suspension of the body. Autopsy confirms death due to partial hanging, where only part of the body weight acts as the constricting force on the neck structures.

During forensic teaching, the residents are asked about the approximate minimum body weight/force required to produce fatal neck compression in partial hanging.

Death in partial hanging is most likely due to application of approximately what minimum weight?

- a) 1.15 kg
- b) 2.20 kg
- c) 3.10 kg
- d) 4.50 kg

Q94. A 25-year-old male presents with high-grade fever, severe throat pain, and difficulty in swallowing for the past 5 days. He also complains of trismus and a muffled "hot potato" voice. On examination, there is medial bulging of the lateral pharyngeal wall with displacement of the tonsil. Neck examination reveals swelling below the angle of mandible.

What is the most likely diagnosis?



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- a) Peritonsillar abscess
- b) Retropharyngeal abscess
- c) Parapharyngeal abscess
- d) Ludwig's angina

Q95. A patient diagnosed with parapharyngeal abscess develops worsening neck swelling, high fever, and signs of sepsis. Imaging reveals extension of infection into deeper neck spaces with involvement of major vascular structures. The clinician is concerned about a life-threatening complication involving the carotid sheath.

Which of the following is the most serious complication associated with this condition?

- a) Facial nerve palsy
- b) Internal jugular vein thrombosis
- c) Otitis media
- d) Nasal septal abscess

Q96. A 32-year-old man presents to the emergency department with sudden onset throat discomfort and difficulty swallowing. He describes a sensation of a "foreign body" in the throat. There is no history of fever. On examination, a markedly swollen, pale, and edematous uvula is seen hanging in the oropharynx. There is no pus or tonsillar involvement. The patient has a history of recent seafood intake.

What is the most likely diagnosis?

- a) Peritonsillar abscess
- b) Acute epiglottitis
- c) Quincke's disease
- d) Retropharyngeal abscess

Q97. A 28-year-old woman presents with sudden onset throat swelling and mild breathing difficulty. Examination reveals a markedly edematous uvula without signs of infection. She has recently taken an NSAID. The clinician suspects allergic uvular edema and plans immediate treatment to prevent airway compromise.

Which of the following is the most appropriate initial management?

- a) Intravenous antibiotics
- b) Emergency tracheostomy
- c) Antihistamines and corticosteroids
- d) Incision and drainage

Q98. A 55-year-old man presents with hoarseness of voice for 2 months. He also reports difficulty in swallowing and occasional choking while eating. There is a history of thyroid surgery 3 months ago. On laryngoscopic examination, the left vocal cord is found to be immobile and lies in the paramedian position.

Which nerve is most likely injured in this patient?

- a) Superior laryngeal nerve (external branch)
- b) Superior laryngeal nerve (internal branch)
- c) Recurrent laryngeal nerve
- d) Glossopharyngeal nerve



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Q99. A 60-year-old chronic smoker presents with progressive hoarseness of voice for 3 months. There is no significant pain or dysphagia. On indirect laryngoscopy, a lesion is seen involving the true vocal cords. There is no cervical lymphadenopathy.

Which of the following is the most likely characteristic of this lesion?

- a) Early lymph node metastasis
- b) Late presentation with airway obstruction
- c) Early symptom of hoarseness with good prognosis
- d) Rapid spread to subglottic region

Q100. A 6-year-old child is brought with complaints of hearing difficulty and inattentiveness in school for the past 3 months. There is no history of ear pain or discharge. Otoscopic examination reveals a dull, retracted tympanic membrane with air-fluid levels visible behind it. Tympanometry shows a type B curve. The child has a history of recurrent upper respiratory tract infections.

What is the most likely underlying mechanism responsible for this condition?

- a) Increased cerumen production
- b) Eustachian tube dysfunction
- c) Perforation of tympanic membrane
- d) Ossicular chain discontinuity

Q101. A 28-year-old male presents with foul-smelling ear discharge for 1 year and progressive hearing loss. On otoscopy, a retraction pocket is seen in the pars flaccida with keratin debris. There is associated erosion of ossicles on imaging.

What is the most likely diagnosis?

- a) Acute suppurative otitis media
- b) Tubotympanic CSOM
- c) Atticoantral disease (cholesteatoma)
- d) Otosclerosis

Q102. A 32-year-old woman presents with slowly progressive bilateral hearing loss. She reports that she hears better in noisy environments. There is no history of ear discharge. Rinne test is negative, and Weber lateralizes to the affected ear. Audiometry shows a dip in bone conduction at 2 kHz.

What is this audiological phenomenon called?

- a) Recruitment
- b) Carhart notch
- c) Diplacusis
- d) Tullio phenomenon

Q103. A 45-year-old female presents with unilateral progressive sensorineural hearing loss and tinnitus. She also has imbalance but no true vertigo. MRI shows a mass at the cerebellopontine angle.

Which nerve is most commonly involved?

- a) Facial nerve
- b) Vestibulocochlear nerve
- c) Trigeminal nerve
- d) Glossopharyngeal nerve



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Q104. A 58-year-old female presents to the emergency department with sudden onset severe pain in her right eye associated with headache and repeated episodes of vomiting. She complains of blurred vision and seeing halos around lights since the onset of symptoms. There is no history of trauma or discharge. On examination, the eye is congested with a hazy cornea, and the pupil is mid-dilated and non-reactive to light. Intraocular pressure is found to be markedly elevated.

What is the most likely diagnosis?

- a) Open-angle glaucoma
- b) Acute angle-closure glaucoma
- c) Uveitis
- d) Keratitis

Q105. A 65-year-old male with a history of long-standing hypertension presents with sudden, painless loss of vision in his right eye since early morning. There is no history of trauma, redness, or ocular discharge. On examination, visual acuity is markedly reduced. Fundoscopic evaluation reveals a pale, edematous retina with a characteristic cherry-red spot at the macula.

What is the most likely diagnosis?

- a) Central retinal vein occlusion
- b) Retinal detachment
- c) Central retinal artery occlusion
- d) Optic neuritis

Q106. A 52-year-old male with a 12-year history of poorly controlled diabetes mellitus presents with gradual, painless diminution of vision in both eyes. There is no history of acute vision loss or ocular pain. Fundoscopic examination reveals multiple microaneurysms, dot-blot hemorrhages, hard exudates, and cotton wool spots. However, there is no evidence of neovascularization at the disc or elsewhere in the retina.

What is the most likely stage of diabetic retinopathy?

- a) Mild non-proliferative
- b) Moderate non-proliferative
- c) Severe non-proliferative
- d) Proliferative retinopathy

Q107. What is the most likely stage of diabetic retinopathy?

- a) Mild non-proliferative
- b) Moderate non-proliferative
- c) Severe non-proliferative
- d) Proliferative retinopathy

Q108. A 28-year-old female presents with sudden onset diminution of vision in her right eye for the past 2 days. She complains of pain that worsens on eye movement. There is no history of redness or discharge. On examination, visual acuity is reduced and a relative afferent pupillary defect (RAPD) is present. Fundoscopic examination appears normal.

What is the most likely associated condition?

- a) Diabetes mellitus
- b) Hypertension
- c) Multiple sclerosis
- d) Myasthenia gravis



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Q109. A 45-year-old male with a history of high myopia presents with sudden onset flashes of light followed by multiple floaters in his right eye. Over the next few hours, he notices a progressive shadow descending over his visual field, described as a “curtain” effect. There is no history of trauma or ocular pain. Fundoscopic examination reveals a detached, corrugated retina with a visible horseshoe-shaped retinal tear.

What is the most likely type of retinal detachment?

- a) Tractional
- b) Exudative
- c) Rhegmatogenous
- d) Serous

Q110. A 20-year-old male presents with gradually progressive diminution of vision over the past 2 years. He reports frequent change in spectacle prescription with poor visual improvement. There is no history of pain, redness, or discharge. Refraction reveals increasing myopia with significant irregular astigmatism. On slit lamp examination, there is localized corneal thinning with a cone-shaped protrusion of the cornea.

What is the most likely diagnosis?

- a) Keratitis
- b) Keratoconus
- c) Corneal ulcer
- d) Pterygium

Q111. A 7-day-old neonate presents with progressive jaundice, characterized by yellowish discoloration of sclera and dark urine. The parents also report passage of clay-colored stools. On examination, hepatomegaly is noted with the liver palpable 7 cm below the right costal margin. Liver function tests suggest obstructive pathology, and ultrasound shows biliary abnormality. A HIDA scan confirms absence of bile flow into the intestine. Surgical management is planned, involving an anastomosis between the jejunum and the porta hepatis to facilitate bile drainage.

Which of the following procedures is most likely planned?

- a) Ramstedt's seromyotomy
- b) Heller's myotomy
- c) Cyst excision with hepaticojejunostomy
- d) Kasai operation

Q112. A patient is brought to the emergency department following a road traffic accident. He has sustained a pelvic fracture and is unable to pass urine. There is blood at the external urethral meatus, raising suspicion of urethral injury. The clinician plans an initial investigation to confirm the diagnosis before attempting catheterization.

Which of the following is the most appropriate initial investigation in this patient?

- a) Micturating cystourethrogram (MCU)
- b) Cystogram
- c) Retrograde urethrogram (RGU)
- d) Intravenous pyelography (IVP)

Q113. A patient presents with a small abnormal opening in the lower part of the neck, located along the anterior border of the sternocleidomastoid muscle. During evaluation, an instrument is introduced through this



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external opening and is traced upwards toward the anterior aspect of the posterior faucial pillar behind the tonsil. This procedure is performed to delineate the tract of a suspected branchial fistula.

Which of the following techniques is being used in this procedure?



- a) Syringing
- b) Probing
- c) Needling
- d) Wiring

Q114. A luxury bus carrying multiple passengers meets with a high-speed accident on the highway. Several victims present with blunt abdominal trauma. Some patients are hemodynamically stable, while others are unstable with signs of internal bleeding. The trauma team plans appropriate imaging and evaluation strategies based on patient stability.

Which of the following statements regarding blunt abdominal injury is correct?

- a) Hepatic injury should not be suspected if there is blunt injury to the lower chest
- b) Should be initially investigated by MRI scanning
- c) Hemorrhage cannot be temporarily controlled by Pringle's maneuver
- d) Best evaluation is by CECT in stable patients and FAST in unstable patients

Q115. A child presents with painless lower gastrointestinal bleeding. Imaging suggests a congenital diverticulum located approximately 2 feet proximal to the ileocecal junction on the antimesenteric border of the ileum. A Technetium-99m pertechnetate scan is performed to aid in diagnosis.

Which type of epithelium is most prominently highlighted in this scan?

- a) Ileal
- b) Colonic
- c) Jejunal
- d) Gastric

Q116. A 25-year-old woman presents with spontaneous bloody discharge from a single duct of the breast. There is no palpable lump, skin changes, or axillary lymphadenopathy. The discharge is unilateral and persistent. Clinical evaluation suggests an intraductal pathology.

What is the most appropriate treatment for this patient?

- a) Microdochectomy
- b) Modified radical mastectomy (MRM)
- c) Observation only
- d) Chemotherapy

Q117. A patient with extensive full-thickness burns is undergoing long-term rehabilitation. Over time, he develops features suggestive of nutritional deficiency. The clinician explains that prolonged immobilization, reduced sun exposure, and metabolic disturbances after burns can affect certain vitamin levels.

Which of the following vitamin deficiencies is most likely to occur as a long-term consequence of such burn injuries?



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- a) Vitamin A
- b) Vitamin B
- c) Vitamin C
- d) Vitamin D

Q118. A 30-year-old man presents to the emergency department with severe right upper quadrant pain, high-grade fever (40°C), anorexia, and nausea. He recently returned from a 3-week stay in a slum area. On examination, there is marked tenderness in the right hypochondrium and a positive “thump sign”. He also complains of cough and referred pain to the shoulder region.

What is the most likely diagnosis?

- a) Chronic cholecystitis
- b) Cholangitis
- c) Amoebic liver abscess
- d) Perforation peritonitis

Q119. A 30-year-old man from Bihar presents with right hypochondrial pain and fever. Ultrasonography reveals a 6 × 5 × 4 cm abscess in the left lobe of the liver. Serology for hydatid disease is negative. The clinical picture is suggestive of amoebic liver abscess. The treating physician plans appropriate management considering the size and location of the lesion.

What is the most appropriate treatment for this patient?

- a) Anti-amoebic drugs
- b) Catheter drainage + anti-amoebic drugs
- c) Aspiration + anti-amoebic drugs
- d) Percutaneous aspiration with scolicedal agents

Q120. An elderly male presents with complaints of calf pain while walking approximately 500 meters, which is relieved by rest. The clinical picture is suggestive of intermittent claudication due to peripheral arterial disease (PAD). He seeks advice regarding modifiable risk factors to reduce disease progression.

Which of the following is the most appropriate advice for this patient?

- a) If he has two risk factors, his risk of arterial disease is decreased
- b) He should not worry about his cholesterol level as this is not a risk factor
- c) His risk will be less as he gets older
- d) Continuing smoking will increase his risk of peripheral arterial disease

Q121. A 20-year-old woman presents with episodic color changes in her fingers on exposure to cold. She describes a sequence of white → blue → red discoloration, associated with numbness and pain. The clinical picture is suggestive of Raynaud’s phenomenon. The physician considers pharmacological options to improve peripheral blood flow.

Which of the following drugs would be least useful in the management of this condition?

- a) Amlodipine
- b) Diltiazem
- c) Nitroglycerine
- d) Tamsulosin



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Q122. A 48-year-old chronic smoker presents with progressive pain in the right foot, calf, and lower leg for several months. The pain worsens on lying supine with elevation of the limb, and he prefers to sleep in a sitting position. On examination, dorsalis pedis and posterior tibial pulses are absent, while the femoral pulse is normal on the affected side. Pulses in the opposite limb are normal.

What is the most likely site of arterial obstruction in this patient?

- a) Anterior and posterior tibial arterial obstruction
- b) Distal femoral arterial obstruction
- c) Iliofemoral arterial obstruction
- d) Aortoiliac arterial obstruction

Q123. A 45-year-old man presents with pain in the right gluteal, buttock, and thigh region while walking, which is relieved on rest. The symptoms have gradually progressed over the past few months. On examination, all pulses in the right lower limb are absent, while pulses in the left lower limb are normal. On auscultation, a bruit is heard over the right iliac region.

What is the most likely site of arterial obstruction in this patient?

- a) Femoral-popliteal occlusion
- b) Aortoiliac occlusion
- c) Iliac artery occlusion
- d) Distal popliteal occlusion

Q124. A 72-year-old man is brought to the emergency department in shock with cold, clammy extremities and absent peripheral pulses. His systolic blood pressure is unrecordable in the upper limbs. There is no history of trauma. Resuscitation is initiated, and there is strong clinical suspicion of massive retroperitoneal hemorrhage with high mortality risk.

Which of the following is the most likely cause of this presentation?

- a) Rupture of pancreatic pseudocyst
- b) Bleeding from hepatic hemangioma
- c) Splenic artery aneurysm rupture
- d) Rupture of abdominal aortic aneurysm

Q125. A 30-year-old man is brought to the emergency department after a penetrating chest injury. He is tachypneic, tachycardic (126 bpm), hypotensive (BP 86/58 mmHg) with cold, clammy extremities. On examination, he appears distressed and there is tracheal deviation. Oxygen has been administered and IV access secured. The clinical picture suggests a life-threatening thoracic emergency requiring immediate intervention.

What is the most appropriate initial management in this patient?

- a) Needle thoracocentesis in the 5th intercostal space midaxillary line followed by chest drain insertion
- b) CT scan of the chest
- c) Bilateral thoracostomy
- d) Chest X-ray

Q126. A woman presents with continuous leakage of clear fluid per vaginum suggestive of urinary incontinence. A dye test is performed by instilling colored fluid into the urinary bladder. However, the vaginal pad does not show any staining, although it remains soaked with clear fluid. This suggests that the source of leakage is not directly from the bladder.

What is the most likely diagnosis?



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- a) Vesicovaginal fistula (VVF)
- b) Uretero-vaginal fistula
- c) Urinary stress incontinence
- d) Urethrovaginal fistula

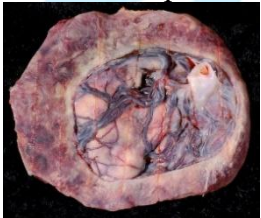
Q127. A 26-year-old primigravida presents to the antenatal clinic for her first visit during the second trimester. She is asymptomatic but reports a family history of thyroid disorders. As part of routine evaluation, thyroid function tests are ordered. The clinician notes that some parameters may be physiologically altered during pregnancy due to hormonal changes.

Which of the following thyroid function test parameters, if altered, is least concerning in pregnancy?

- a) Free T3 level
- b) Free T4 level
- c) Total T3 level
- d) TSH level

Q128. A primigravida delivers a term baby with a birth weight of 1.9 kg following an uncomplicated vaginal delivery. On inspection of the placenta after delivery, the obstetrician notices a central depressed area with a thickened, raised ring at the margins, giving the placenta a rolled edge appearance. There is no evidence of accessory lobes. The finding is associated with fetal growth restriction.

What is this placental condition known as?



- a) Placenta succenturiate
- b) Placenta membranacea
- c) Circumvallate placenta
- d) Circummarginate placenta

Q129. A 30-year-old female presents to the OPD with a confirmed twin pregnancy at 10 weeks of gestation. She has a history of one full-term vaginal delivery, resulting in a healthy 3-year-old child. Additionally, she reports two first-trimester spontaneous abortions in the past. There is no history of preterm delivery or stillbirth. The clinician is asked to calculate her current obstetric score using the GTPAL system.

What is the correct obstetric score?

- a) G4 T0 P2 L1 A2
- b) G4 T1 P0 A2 L1
- c) G5 T2 P0 L1 A2
- d) G5 T4 P1 A2 L1

Q130. A 32-year-old diabetic woman in active labour delivers the fetal head after a prolonged second stage. However, there is difficulty in delivering the shoulders, and the head retracts against the perineum ("turtle sign"). The head-to-shoulder delivery interval exceeds 60 seconds, raising concern for shoulder dystocia. Immediate intervention is required to prevent fetal hypoxia and birth trauma.



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What is the first manoeuvre that should be performed to facilitate delivery?

- a) Rubin manoeuvre
- b) McRoberts manoeuvre
- c) Zavanelli manoeuvre
- d) Wood corkscrew manoeuvre

Q131. A primigravida presents for her first antenatal visit at 14 weeks of gestation with complaints of excessive nausea and vomiting. On abdominal examination, the uterine size appears larger than expected for gestational age, with the fundal height midway between the pubic symphysis and umbilicus. Ultrasound examination reveals a “T sign” at the intertwin membrane insertion. There is no evidence of fused fetal parts.

What is the most likely diagnosis?

- a) Dichorionic diamniotic twins
- b) Monochorionic diamniotic twins
- c) Monochorionic monoamniotic twins
- d) Siamese twins

Q132. A 30-year-old woman is brought to the emergency department in a drowsy state with complaints of severe lower abdominal pain and spotting per vaginum. She gives a history of amenorrhoea for 8 weeks, and her urine pregnancy test is positive. On examination, she is hypotensive (BP: 80/60 mm Hg) with signs of circulatory shock. Abdominal examination reveals guarding and tenderness.

What is the next step in management?

- a) Monitor serum β -hCG levels after one week
- b) Resuscitation and emergency laparotomy with salpingectomy
- c) Suction and evacuation
- d) Laparoscopic salpingostomy

Q133. A 36-year-old multiparous woman at 32 weeks of gestation presents with recurrent episodes of painless, bright red bleeding per vaginum. She had a previous lower segment cesarean section in her last pregnancy. On examination, the uterus is soft, non-tender, and fundal height corresponds to gestational age. Fetal heart rate is 140 bpm, and fetal parts are easily palpable. The clinical picture is suggestive of placenta previa.

Which of the following is NOT a risk factor for this condition?

- a) Pre-eclampsia
- b) Multifetal gestation
- c) Cigarette smoking
- d) Previous history of similar condition

Q134. A 24-year-old primigravida at 32 weeks of gestation presents with regular uterine contractions and cervical changes, consistent with preterm labour. There is no evidence of fetal distress or contraindication to prolonging pregnancy. The goal is to delay delivery to allow administration of corticosteroids for fetal lung maturity. The clinician plans to initiate tocolytic therapy.

Which of the following is the preferred tocolytic agent in this case?

- a) Nifedipine
- b) Glyceryl trinitrate
- c) Ritodrine
- d) Magnesium sulphate



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Q135. A 22-year-old primigravida at 14 weeks of gestation presents with excessive vomiting and intermittent vaginal bleeding for the past 2 weeks. On examination, the uterine size is larger than expected (≈ 16 weeks), and fetal heart sounds are not detectable. Ultrasonography reveals a “snowstorm appearance” without identifiable fetus. The patient is hemodynamically stable.

What is the next step in management?

- a) Chemotherapy
- b) Total abdominal hysterectomy
- c) Suction and evacuation
- d) Serum β -hCG estimation after one week

Q136. A 32-year-old woman presents with complaints of continuous dribbling of urine per vaginum, especially noticeable during and after micturition. There is no associated pain or dysuria. On examination, a methylene blue three-swab test is performed. The lowest vaginal swab is stained blue, while the upper swabs remain unstained.

What is the most likely diagnosis?

- a) Uterovaginal fistula
- b) Vesicovaginal fistula
- c) Urethrovaginal fistula
- d) Vesicouterine fistula

Q137. A 45-year-old multiparous woman presents with complaints of heavy menstrual bleeding (menorrhagia) and progressive secondary dysmenorrhea over the past year. On pelvic examination, the uterus is found to be symmetrically enlarged, soft, and tender, suggestive of a “boggy uterus.” Halban’s sign is positive, and transvaginal ultrasonography shows a characteristic “salt and pepper” appearance of the myometrium.

What is the definitive treatment for this condition?

- a) Continuous GnRH therapy
- b) Total abdominal hysterectomy
- c) Progesterone therapy
- d) Cauterisation

Q138. A 27-year-old woman presents with primary infertility of 3 years duration. She has regular menstrual cycles and no significant past medical history. As part of her evaluation, hysterosalpingography (HSG) is performed, which shows bilateral cornual (proximal tubal) block. She is otherwise asymptomatic, and there are no clinical features suggestive of pelvic infection.

What is the next step in further evaluation?

- a) Start anti-tubercular therapy
- b) Hysteroscopy and laparoscopy
- c) Recanalisation
- d) In-vitro fertilization

Q139. A 67-year-old postmenopausal woman presents with chronic vulvovaginal itching and irritation for several months. On examination, there are areas of vulvar discoloration with irregular epithelial thickening, raising suspicion of vulvar intraepithelial neoplasia or carcinoma. She has no history of sexually transmitted infections but reports long-standing symptoms. A biopsy is planned for confirmation.



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What is the most common etiological factor associated with this condition?

- a) HPV 16
- b) HPV 18
- c) HPV 6
- d) Chlamydia

Q140. An 18-year-old female presents with primary amenorrhea. She has normal height and well-developed secondary sexual characteristics, including normal breast development. There is no history of cyclic abdominal pain. On examination, external genitalia appear normal female. Pelvic ultrasonography reveals normal ovaries, but absence of uterus and cervix. Hormonal profile is within normal female range. Karyotyping shows 46,XX genotype.

What is the most likely diagnosis?

- a) Pure gonadal dysgenesis
- b) Müllerian agenesis
- c) Testicular feminisation syndrome
- d) Swyer syndrome

Q141. 19-year-old adolescent girl presents with primary amenorrhea and cyclical lower abdominal pain for the past 4–5 months. She reports normal pubertal development, including well-developed breasts and pubic hair. There is no history of menstrual bleeding since menarche age. On examination, a tense, bulging vaginal mass is suspected. Pelvic ultrasonography reveals hematocolpos (collection of menstrual blood in the vagina).

Which of the following conditions is NOT a cause of this presentation?

- a) Transverse vaginal septum
- b) Vaginal atresia
- c) Septate uterus
- d) Imperforate hymen

Q142. A 12-year-old child presents with fever, joint pains, and malaise for the past 1 week. There is a history of sore throat 3 weeks ago which was not treated adequately. On examination, the child is febrile, and migratory arthritis is suspected. Laboratory investigations reveal elevated inflammatory markers. The clinician considers Modified Jones Criteria for diagnosing acute rheumatic fever.

Which of the following represents a minor criterion in this diagnostic system?

- a) ASO titre
- b) Past history of rheumatic fever
- c) Fever
- d) Subcutaneous nodules

Q143. A 42-year-old male, chronic smoker, is brought to the emergency department after a road traffic accident (fall from bike). He complains of sudden onset left-sided chest pain and acute breathlessness. On examination, he is tachycardic (HR 110/min) with SpO₂ of 72% on room air and BP 150/88 mmHg. Inspection reveals reduced chest expansion on the left side. On auscultation, there are absent breath sounds and absent vocal fremitus over the left hemithorax.

What is the most likely diagnosis?

- a) Acute Respiratory Distress Syndrome



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- b) Acute exacerbation of COPD
- c) Pleural effusion
- d) Pneumothorax

Q144. A 68-year-old male presents with fever with chills, progressive shortness of breath, and cough for the past 7 days. He reports expectoration of dark, chocolate-colored sputum. He has a long occupational history as a transporter and appears chronically ill. On examination, he is tachycardic (HR 112/min), hypotensive (BP 90/58 mmHg), and hypoxic (SpO₂ 90%). Systemic examination reveals axillary lymphadenopathy, and chest auscultation shows bilateral interscapular crepitations.

Which of the following is the most likely causative organism?

- a) *Klebsiella pneumoniae*
- b) *Pseudomonas aeruginosa*
- c) *Pneumocystis jirovecii*
- d) *Staphylococcus aureus*

Q145. A 55-year-old asymptomatic male presents for a routine health check-up. He has no significant complaints but is concerned about cancer risk due to family history. The physician discusses various screening programs and their impact on overall mortality reduction. It is explained that not all screening tests improve survival, as some may lead to lead-time bias or overdiagnosis without true mortality benefit.

Which of the following cancers has definitive evidence that screening improves overall survival?

- a) Prostate cancer
- b) Lung cancer
- c) Colon cancer
- d) Ovarian cancer

Q146. A 24-year-old female presents with colicky abdominal pain, hematuria, and joint pains for the past few days. She also reports a recent upper respiratory infection. On examination, there is a palpable purpuric rash over the trunk, lower limbs, and buttocks. Her vital signs are stable, and there is no evidence of active bleeding. Laboratory investigations reveal normal coagulation profile with no clotting abnormalities.

What is the most probable diagnosis?

- a) Hemolytic uremic syndrome
- b) Henoch–Schönlein purpura
- c) Meningococcemia
- d) Thrombotic thrombocytopenic purpura

Q147. A 35-year-old male presents with low-grade fever and persistent cough for the past 2 months. He also complains of mild fatigue and weight loss. Chest X-ray reveals bilateral upper lobe fibrosis along with bilateral hilar lymphadenopathy. There is no significant sputum production or hemoptysis. On examination, respiratory findings are minimal. The clinician suspects a granulomatous systemic disease.

Which of the following is the best initial investigation to support the diagnosis?

- a) Mantoux test
- b) Serum ACE levels
- c) Spirometry
- d) Sputum examination



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Q148. A 16-year-old girl presents with recurrent episodes of mucocutaneous bleeding, including epistaxis and menorrhagia since early adolescence. There is no history of trauma or drug intake. On examination, multiple ecchymotic patches are noted. Laboratory evaluation reveals prolonged bleeding time (BT) and prolonged activated partial thromboplastin time (aPTT), while prothrombin time (PT) is normal.

Which of the following conditions is NOT consistent with these findings?

- a) Hemophilia A
- b) Factor VII deficiency
- c) Von Willebrand disease]
- d) Factor XII deficiency

Q149. A 28-year-old male presents with chronic low back pain and hip stiffness, gradually progressive over the past several months. He also complains of exertional shortness of breath and episodes of painful red eyes with decreased vision. There is no history of small joint involvement. On examination, spinal mobility is restricted. Laboratory investigations reveal a negative rheumatoid factor.

Which of the following is the most likely diagnosis?

- a) Sarcoidosis
- b) Ankylosing spondylitis
- c) Rheumatoid arthritis
- d) None of the above

Q150. A 1.5-year-old child is brought with progressive neurodevelopmental regression, irritability, and stiffness of limbs. Parents report excessive crying and feeding difficulty. On examination, there is hypertonia with exaggerated reflexes. MRI brain reveals extensive involvement of deep white matter along with bilateral hyperintense lesions in the thalamus. A hereditary demyelinating disorder is suspected.

Which of the following is the most likely diagnosis?

- a) Alexander disease
- b) Krabbe disease
- c) Canavan disease
- d) Metachromatic leukodystrophy

Q151. A 3-year-old child is brought with complaints of progressive irritability, poor feeding, and abdominal distension. On examination, a firm abdominal mass is palpable crossing the midline. There is associated weight loss and intermittent fever. Pediatric oncology evaluation is planned. The clinician discusses common childhood malignancies and emphasizes the most frequent solid extracranial tumor seen in children.

Which of the following is the most common solid malignancy in children?

- a) Brain tumor
- b) Neuroblastoma
- c) Wilms tumor
- d) Sacrococcygeal teratoma

Q152. A 6-month-old infant is brought for a routine developmental assessment. The child is able to sit with support using hands (tripod position) and shows interest in surroundings. The mother reports that the baby produces monosyllabic sounds and smiles when looking into a mirror. On examination, the child is socially



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interactive and visually attentive. The pediatrician evaluates whether the developmental milestones are appropriate for age.

Which of the following is NOT expected at 6 months of age?

- a) Pincer grasp
- b) Monosyllable sounds
- c) Watching self in mirror
- d) Sitting in tripod position

Q153. A 10-year-old boy is brought for evaluation of short stature. His height is below the 3rd percentile for age, but his growth velocity has remained normal over the past few years. There are no systemic complaints, and developmental milestones are normal. On evaluation, his chronological age is greater than his bone age. Family history reveals delayed puberty in one of the parents.

What is the most likely diagnosis?

- a) Constitutional delay of growth and puberty
- b) Genetic (familial) short stature
- c) Hypopituitarism
- d) Primordial dwarfism

Q154. A 2-year-old child is brought with progressive pallor, irritability, and poor feeding. There is no history of acute blood loss. Family history reveals similar illness in a sibling. On examination, the child appears pale with mild hepatosplenomegaly. Laboratory investigations show hemoglobin of 6 g/dL with microcytic hypochromic anemia. Peripheral blood smear demonstrates target cells.

What is the next best investigation to confirm the diagnosis?

- a) Hb electrophoresis
- b) Coombs test
- c) Liver function tests
- d) Osmotic fragility test

Q155. A young child is brought with complaints of hair loss (alopecia) and chronic skin lesions. On examination, there is hyperpigmentation with psoriasiform dermatitis, particularly involving the perioral, genital, and mucosal regions. The child also shows features of delayed sexual development (hypogonadism). There is no history of systemic illness. Nutritional deficiency is suspected.

Which of the following is the most likely diagnosis?

- a) Copper deficiency
- b) Iron deficiency
- c) Zinc deficiency
- d) Magnesium deficiency

Q156. A 4-month-old infant is brought with recurrent seizures and an erythematous skin rash. The child is lethargic and feeding poorly. Arterial blood gas analysis shows metabolic acidosis, and laboratory evaluation reveals elevated ketone levels. There is no history suggestive of liver failure. An inherited metabolic disorder affecting biotin-dependent enzymes is suspected.

Which of the following is the most likely diagnosis?

- a) Propionic aciduria
- b) Urea cycle disorder



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- c) Phenylketonuria
- d) Multiple carboxylase deficiency

Q157. A full-term newborn is delivered vaginally and is vigorous at birth. The baby cries immediately and has good tone. On examination, the neonate is covered with blood, amniotic fluid, and a whitish cheesy substance over the skin. The attending nurse prepares for routine newborn care and cleaning. The pediatrician advises selective removal of substances from the baby's skin.

Which of the following need not be routinely removed immediately after birth?

- a) Blood
- b) Meconium
- c) Vernix caseosa
- d) None

Q158. A very preterm neonate on enteral feeds (30 mL/kg/day) develops sudden abdominal distension on day 6 of life. The abdomen appears tense with visible bowel loops. The baby also shows temperature instability and lethargy, suggestive of systemic involvement. On evaluation, abdominal X-ray reveals portal venous gas. There is no evidence of pneumoperitoneum.

According to staging of necrotizing enterocolitis, what is the stage of this condition?

- a) Stage IB
- b) Stage IIA
- c) Stage IIB
- d) Stage IIIA

Q159. A 7-month-old infant is brought for a routine developmental assessment. The mother reports that the child is active and playful. On examination, the infant is able to sit with support, transfer objects from one hand to another, and shows interest in surroundings, including playing with his reflection in the mirror. The pediatrician assesses whether all milestones are appropriate for age.

Which of the following milestones is NOT expected at 7 months of age?

- a) Sitting with support
- b) Walking with support
- c) Transferring objects
- d) Mirror play

Q160. A 32-week-old. On further dissection, there is evidence of soot deposition in deeper tissues. The forensic expert with APGAR scores of 5 and 6 at 1 and 5 minutes is admitted to the NICU. On day 5 of life, the baby becomes hypoactive and develops progressive abdominal distension. There is blood in stools, and feeding intolerance is noted. On examination, the abdomen is tense. Abdominal X-ray reveals gas within the bowel wall.

What is the most likely diagnosis?

- a) Necrotizing enterocolitis
- b) Meconium ileus
- c) Hirschsprung disease
- d) Neonatal sepsis



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Q161. A 65-year-old female presents after a fall at home with severe right hip pain and inability to stand. On examination, the affected limb is externally rotated, with the lateral border of the foot resting on the bed. There is ecchymosis over the hip, and active straight leg raise (SLR) is not possible. The patient is hemodynamically stable. Clinical features suggest a fracture around the hip region.

What is the treatment of choice for this injury pattern?

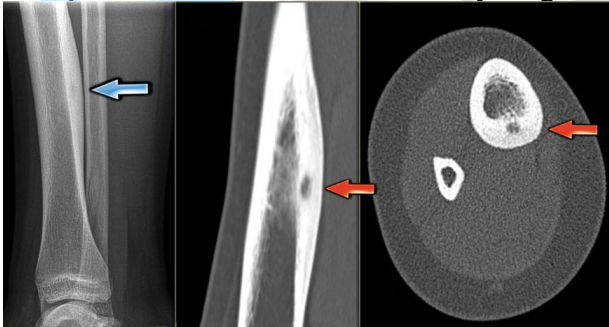
- a) Closed reduction and internal fixation with cancellous screws
- b) Closed reduction and internal fixation with DHS or PFN
- c) Bipolar hemiarthroplasty
- d) Total hip arthroplasty

Q162. A patient undergoes surgical fixation of a proximal radial shaft fracture using a posterior (Thompson's) approach. In the immediate postoperative period, the patient complains of inability to extend the fingers and thumb. On examination, wrist extension is relatively preserved, but there is loss of finger and thumb extension without sensory deficits.

Which of the following is the most likely cause of this deficit?

- a) Injury to common extensor origin
- b) Injury to posterior interosseous nerve
- c) Injury to radial nerve (high palsy)
- d) Injury to anterior interosseous nerve

Q163. What is the most likely diagnosis ?



- a) Osteoid Osteoma
- b) Brodie's Abscess
- c) Ewing's Sarcoma
- d) Simple Bone Cyst

Q164. A 45-year-old patient presents with pain over the radial side of the wrist, especially around the radial styloid. The pain worsens while performing activities such as gripping, lifting, or thumb movements. There is localized tenderness over the first dorsal compartment. The clinician performs a bedside maneuver by asking the patient to flex the thumb into the palm and ulnarly deviate the wrist, which reproduces severe pain.

Which of the following clinical tests is used to diagnose this condition?



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- a) Cozen's test
- b) Finkelstein's test
- c) Reverse Cozen's test
- d) Phalen's test

Q165. A 7-year-old boy presents with acute pain in the right hip and restricted range of movements. He is able to partially bear weight but appears uncomfortable. On examination, the affected limb is held in abduction and external rotation, suggestive of joint effusion. There is no high-grade fever. Laboratory investigations reveal normal hemogram with mildly elevated ESR.

What is the next best step in management?

- a) IV antibiotics and splintage
- b) Watchful observation
- c) USG-guided aspiration
- d) Immediate hospitalization

Q166. 45 year old lady reported to clinic with sudden increase in size of lesion which was present from her childhood without any change of size. identify lesion :



- a) Squamous Cell carcinoma
- b) basal cell carcinoma
- c) Benign melanocytic lesion
- d) Malignant melanoma

Q167. A patient comes with the following lesion on his face. The lesions are scarring in the centre and increasing on the periphery and giving apple jelly nodule on diascopy The most appropriate treatment for this would be?





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- a) Oral steroids
- b) Anti leprosy treatment
- c) Anti TB treatment
- d) Oral terbinafine

Q168. A 34-year-old truck driver presents with the following lesion on his legs which is progressing in size. His CD4 count is 350. Which of the following is the most correct statement associated with this condition?



- a) HHV7- purpura
- b) HHV7- Kaposi sarcoma
- c) HHV8- Kaposi sarcoma
- d) HHV8- purpura

Q169. A patient presents with a localized skin infection following minor trauma. The immune response is initiated at the level of the skin. Specialized cells capture and process the invading antigens and migrate to regional lymph nodes to activate T-lymphocytes. These cells are primarily located in the epidermis and play a crucial role in cutaneous immune surveillance.

Which of the following cells are responsible for antigen processing in the skin?

- a) Fibroblasts
- b) Mast cells
- c) Langerhans cells
- d) Macrophages

Q170. A young adult presents with multiple erythematous, oval to annular skin lesions over the trunk and proximal limbs. The lesions show a fine peripheral collarette of scales with central clearing. The rash appeared gradually and is mildly pruritic. On further questioning, the patient recalls a single larger patch appearing before the generalized eruption.

Which of the following is the most likely diagnosis?

- a) Pityriasis versicolor
- b) Pityriasis rubra pilaris
- c) Pityriasis rosea
- d) Psoriasis

Q171. A 30-year-old man was recently started on high-dose haloperidol for a psychiatric condition. Within a few days, he develops high-grade fever, severe muscle rigidity, akinesia, and mutism. He is diaphoretic, tachycardic, and hypertensive. Laboratory investigations reveal leukocytosis and markedly elevated creatine phosphokinase (CPK) levels. There is no evidence of infection or other drug use.

What is the most likely diagnosis?

- a) Drug overdose



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- b) Neuroleptic malignant syndrome
- c) Drug-induced Parkinsonism
- d) Tardive dyskinesia

Q172. A 25-year-old male met with a road traffic accident (RTA) about 6 weeks ago where he narrowly escaped death. Since then, he has been experiencing recurrent intrusive memories (flashbacks) of the accident, marked fear while driving or passing the accident site, and avoidance of driving altogether. He has also stopped going to work due to distress and anxiety related to the event. His symptoms have persisted continuously for the past 6 weeks and are causing significant occupational dysfunction. What is the most likely diagnosis?

- a) Post-traumatic stress disorder
- b) Acute stress reaction
- c) Depression
- d) Acute panic disorder

Q173. A 30-year-old female presents to the psychiatry OPD with persistent distress following a severe road traffic accident 4 months ago in which she witnessed multiple deaths. She reports recurrent intrusive memories, nightmares, avoidance of driving, and hypervigilance. Her symptoms are affecting her occupational and social functioning. A diagnosis of post-traumatic stress disorder is made. The psychiatrist discusses evidence-based treatment options, including psychotherapies and pharmacotherapy. Which is the most effective first-line psychological treatment modality for this condition?

- a) Cognitive behavioral therapy
- b) Hypnosis
- c) Rational and emotional therapy
- d) Eye movement desensitization and reprocessing

Q174. A 24-year-old female presents to psychiatry OPD with a history of excessive handwashing behavior for the past 2 years. She reports washing her hands nearly 50–60 times per day, driven by an intense fear of contamination despite knowing it is excessive. She experiences significant anxiety if she tries to resist the behavior. Her daily functioning is impaired, and she avoids touching public surfaces. A diagnosis of Obsessive-compulsive disorder is made.

The psychiatrist plans Cognitive Behavioral Therapy (CBT) as part of management. Which CBT technique is the most essential component in her treatment?

- a) Exposure
- b) Response prevention
- c) Thought stopping
- d) Desensitization

Q175. A 15 year boy is not able to get good grades in 9th standard exam. But he is very sharp and intelligence. Best test to diagnose his problem:



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- a) Bhatia's battery
- b) Specific learning disability
- c) Child behavior checklist
- d) Child behavior battery

Q176. A 27-year-old woman is brought to the emergency department following a road traffic accident. She is hemodynamically stable but is evaluated for possible intra-abdominal injury. A FAST (Focused Assessment with Sonography in Trauma) scan is performed, which does not reveal any free fluid in the abdomen. The resident explains that FAST has a threshold below which small amounts of intraperitoneal fluid may not be detected.

What is the approximate minimum volume of fluid that can be detected by FAST scan?

- a) 10 mL
- b) 50 mL
- c) 100 mL
- d) 200 mL

Q177. A young man presents with acute onset cough and fever. Chest X-ray is performed, which shows obliteration of the right heart border while the diaphragm outline remains clearly visible. The clinician uses the silhouette sign to localize the site of lung pathology.

Involvement of which lung lobe is most likely responsible for this radiological finding?

- a) Right lower lobe
- b) Right upper lobe
- c) Right middle lobe
- d) Left lingular lobe

Q178. A 2-year-old boy is brought to the emergency department with sudden onset stridor and respiratory distress. The symptoms began abruptly while he was playing. On chest examination, there are decreased breath sounds with wheeze predominantly on the right side. Chest X-ray reveals an opaque right hemithorax. There is no prior history of fever or upper respiratory infection.

What is the most likely diagnosis?

- a) Pneumothorax
- b) Acute epiglottitis
- c) Massive pleural effusion
- d) Foreign body aspiration

Q179. A young male presents with localized pain and swelling over the right wrist joint, gradually increasing over time. There is no history of trauma or systemic illness. On examination, there is tenderness with restriction of joint movement. Radiographs of bilateral wrist joints reveal an expansile lytic lesion involving the epiphysis of the distal radius with cortical thinning and a "soap-bubble" appearance.

What is the most likely diagnosis?



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- a) Aneurysmal bone cyst
- b) Giant cell tumour
- c) Chondroblastoma
- d) Brown tumor

Q180. A woman undergoing evaluation for infertility is scheduled for a hysterosalpingogram (HSG). During the procedure, radiopaque contrast is injected into the uterine cavity to assess tubal patency. Subsequent radiographs reveal free bilateral spillage of contrast medium into the peritoneal cavity. There are no filling defects or structural abnormalities noted in the uterus.

What does this finding most likely indicate?

- a) The uterine tubes are normal
- b) The mesonephric ducts failed to form properly
- c) The paramesonephric ducts failed to form properly
- d) There is a rectouterine fistula

Q181. A 64-year-old male with a long-standing history of Chronic Obstructive Pulmonary Disease presents to the emergency department with acute exacerbation characterized by dyspnea, wheezing, and productive cough. On examination, he is tachypneic with SpO₂ of 84% on room air. Arterial blood gas shows PaCO₂ of 60 mmHg and pH of 7.32. The attending anesthesiologist decides to initiate controlled oxygen therapy to avoid worsening hypercapnia.

Which of the following oxygen delivery systems is most appropriate in this patient?

- a) Non-rebreather mask (15 L/min)
- b) Nasal cannula (1–2 L/min)
- c) Venturi mask (24–28% FiO₂)
- d) Simple face mask (6–8 L/min)

Q182. A 45-year-old patient is admitted to the ICU following major abdominal surgery and is placed on mechanical ventilation with FiO₂ of 0.9 for prolonged hypoxemia. After 48 hours, the patient develops worsening oxygenation, diffuse bilateral infiltrates on chest imaging, and decreased lung compliance.

Which of the following is the most likely mechanism responsible for this deterioration?

- a) Nitrogen washout leading to atelectasis
- b) Absorption atelectasis due to high oxygen concentration
- c) Oxygen free radical-mediated lung injury
- d) Decreased surfactant production due to hypoxia

Q183. A 28-year-old male is brought to the emergency department following a road traffic accident. He requires rapid sequence intubation. The anesthesiologist administers Succinylcholine for muscle



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relaxation. Shortly after administration, the patient develops generalized muscle fasciculations followed by flaccid paralysis. His serum potassium rises rapidly, and ECG shows peaked T waves.

Which of the following best explains the mechanism of this complication?

- a) Competitive antagonism at nicotinic receptors
- b) Persistent depolarization of the motor end plate
- c) Inhibition of acetylcholine release
- d) Blockade of sodium channels at the neuromuscular junction

Q184. A 52-year-old male is undergoing elective laparotomy under general anesthesia. Midway through the procedure, there is an unexpected failure of oxygen supply from the central pipeline. The anesthesiologist notices that the flow of nitrous oxide automatically decreases and eventually stops, preventing delivery of a hypoxic gas mixture to the patient.

Which of the following safety mechanisms in the anesthesia machine is responsible for this feature?

- a) Oxygen flush valve
- b) Pin Index Safety System
- c) Fail-safe valve (oxygen pressure failure device)
- d) Rotameter flowmeter

Q185. A 26-year-old female is undergoing induction of general anesthesia for elective laparoscopic surgery. Shortly after administration of inhalational anesthetic, she becomes delirious, exhibits irregular breathing, involuntary movements, and exaggerated reflexes including breath-holding and laryngospasm. The anesthesiologist deepens anesthesia to prevent complications.

Which stage of general anesthesia is the patient most likely experiencing?

- a) Stage I – Analgesia
- b) Stage II – Excitement
- c) Stage III - Surgical Anesthesia
- d) Stage IV – Medullary paralysis

Q186. A researcher compares two vaccines:

- Vaccine A produces strong immunity with a single dose but is contraindicated in immunocompromised patients.
- Vaccine B is safer but requires multiple booster doses

Which of the following best matches these vaccines?

- a) A: Killed, B: Live
- b) A: Live attenuated, B: Killed
- c) A: Toxoid, B: Subunit
- d) A: Recombinant, B: Live

Q187. A 30-year-old man with sedentary lifestyle with BP at upper normal limit is advised regular exercise, dietary modification, and stress reduction to prevent progression to overt hypertension. He is not started on medication at this stage. Which component of prevention is most appropriate?

- a) Specific protection
- b) Health promotion
- c) Secondary prevention



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d) Disability limitation

Q188. A 1-year-old child visits a health facility for treatment of mild diarrhoea. The child has missed the measles vaccine scheduled at 9 months. The healthcare provider treats the diarrhoea but does not administer any vaccine, asking the mother to return later for immunization. What is the most appropriate interpretation of this scenario?

- a) Contraindication to vaccination
- b) Proper immunization practice
- c) Missed opportunity for immunization
- d) Vaccine failure

Q189. Group of people coming together for common reason without a leader is called as:

- a) Crowd
- b) Herd
- c) Mob
- d) Society

Q190. Under LaQshya program new instruments and machinery is ordered for better labour room quality services. Which component of primary health care is taken care of:



- a) Community participation
- b) Inter sectorial coordination
- c) Equitable distribution
- d) Appropriate technology

Q191. Incorrect about given method of health education:



- a) Best method to change health attitude and behaviour
- b) 6-12 experts discuss among themselves
- c) Open ended questions preferred
- d) Sociogram is prepared



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Q192. Government is planning to invest 1000 crore Rs for children care with aim to reduce IMR, it is an example of:

- a) Cost benefit analysis
- b) Cost effective analysis
- c) Network analysis
- d) Work sampling

Q193. 55year man reported to ophthalmic OPD with C/O of improper vision. On examination vision in Rt eye- < 6/60, LT eye-1/60, His stage is classified as:

- a) No visual impairment
- b) Mild to moderate visual impairment
- c) Severe visual impairment
- d) Blindness

Q194. You are Medical Officer sitting in OPD, suddenly 15year old boy came with his mother with wound given in image following street dog bite. Mother is giving H/O full vaccination 1year back following same episode. How you will manage this case?



- a) Only wound wash as received vaccine earlier
- b) Give full vaccination – 5 doses as day 0,3,7,14,28
- c) Give only 2 booster dose- day 0,3
- d) Give 2 booster dose with immunoglobulin

Q195. 25year XXXX lady who is a known case of HIV is pregnant now and advised to continue her medication in pregnancy. Under PPTCT program guidelines best prophylaxis for her newborn baby as she is planning to breast feed this baby:

- a) No medication needed as mother has taken ART in full pregnancy duration
- b) Single dose of nevirapine within 72 hours of delivery as mother has taken ART in full pregnancy duration
- c) Nevirapine daily for 6weeks, to be started within 72hours of delivery as mother has taken ART in full pregnancy duration
- d) Nevirapine and zidovudine both for 12 weeks, to be started within 72hours of delivery as she is planning for breast feed her baby

Q196. Incorrect about TB treatment guidelines:

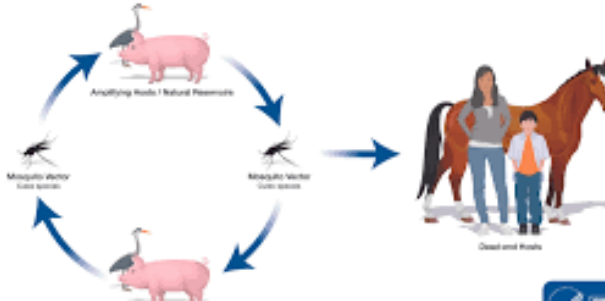
- a) 2month intensive phase HRZE for new case of drug sensitive TB
- b) 4month continuous phase HRE for new case of drug sensitive TB
- c) 2month continuous phase HRZE, 4month intensive phase HRE for new case of drug sensitive TB
- d) 6month BPALM treatment for MDR cases

Q197. Incorrect about disease given in image:



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- a) Pigs- amplifiers but does not develop disease
- b) Vector – mosquito breeds in dirty water
- c) Cattle – mosquito attractant
- d) UIP vaccination- routine vaccination at 9month in all over India with KOLAR strain vaccine

Q198. Diseases covered under IMNCI guidelines are all except:

- a) Measle
- b) Malaria
- c) Malnutrition
- d) Mumps

Q199. All are correct about leprosy program except:



- a) SAPNA
- b) Program is national leprosy elimination program
- c) ASHA is main worker for active surveillance- ABSULS
- d) 3 drugs therapy for both pauci and multi bacillary leprosy

Q200. Correct about given program logo:



- a) Cash incentive of 5000Rs for primigravida after hospital delivery
- b) Free ANC care on 9th of every month at govt sector
- c) Free ANC care on 9th of every month at private sector
- d) Cash incentive of 1400Rs is delivery in rural low performance states



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ANSWERS

1	c
2	b
3	b
4	a
5	d
6	a
7	b
8	a
9	c
10	c
11	c
12	c
13	c
14	d
15	c
16	b
17	c
18	c
19	b
20	b
21	c
22	c
23	d
24	b
25	c
26	c
27	b
28	b
29	c
30	c
31	b
32	c
33	b
34	c
35	a
36	b
37	b
38	c
39	c
40	c
41	b

42	b
43	b
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45	c
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47	b
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50	c
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52	c
53	b
54	c
55	b
56	b
57	d
58	d
59	b
60	b
61	d
62	c
63	c
64	c
65	a
66	b
67	d
68	a
69	d
70	c
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74	d
75	a
76	d
77	b
78	b
79	a
80	d
81	d
82	c

83	d
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124	d
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132	b
133	a
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135	c
136	c
137	b
138	b
139	a
140	b
141	c
142	c
143	d
144	c
145	c
146	b
147	b
148	b
149	b
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151	b
152	a
153	a
154	a
155	c
156	d
157	c
158	c
159	b
160	a
161	b
162	b
163	a
164	b

165	c
166	d
167	c
168	c
169	c
170	c
171	b
172	a
173	a
174	b
175	b
176	d
177	c
178	d
179	b
180	a
181	c
182	c
183	b
184	c
185	b
186	b
187	b
188	c
189	a
190	d
191	b
192	b
193	c
194	c
195	c
196	c
197	d
198	d
199	b
200	c



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